

CONCLUSION

What Vermont Proves, and What Remains to Be Proven

The Argument, Restated

This book opened with a room full of people in Vermont learning, with unusual specificity and unusual honesty, exactly how close their healthcare system was to a financial cliff. Oliver Wyman's September 2024 presentation named the numbers — nine hospitals losing money, thirteen of fourteen projecting losses by 2028, \$2.4 billion in cumulative five-year deficit under realistic assumptions — and drew the causal chain from commercial cross-subsidy to demographic shift to premium inflation to the household economics that make healthcare the dominant financial anxiety of working Vermont families. The room was full because people already knew something was wrong. The presentation gave that feeling a precise shape.

The book's central argument has been this: Vermont's crisis is structurally produced, and structural crises require structural responses. The fifteen chapters between that opening room and this conclusion have developed what structural response means across six pillars — Policy, Economics, Technology, Clinical, Equity, and Operations — and have shown, through Vermont's specific legislative and administrative record, what structural response looks like when it is actually attempted rather than perpetually deferred.

The argument is not that Vermont has solved its healthcare crisis. It has not. As of April 2026, reference-based pricing has not yet taken effect. Global budgets are still being designed. The Statewide Strategic Plan has not been written. The analytics vendor has not been selected. The CCBHC network is still being certified. The Northeast Kingdom hospitals are still financially fragile. Vermont is mid-transformation — past the point of no return on the statutory timeline but nowhere near the destination. The argument is that Vermont has committed to structural change with sufficient specificity, sufficient statutory authority, and sufficient capital investment that the transformation has a credible path to completion. That is more than any comparable American state has done.

What the Six Pillars Prove

The six-pillar framework is this book's primary analytical contribution, and its validity deserves one final statement. The framework is not a consulting taxonomy or a strategic planning template. It is a description of the architecture of healthcare transformation — the six domains that must all function for transformation to produce its promised results, and the specific dependencies between them that explain why single-pillar interventions consistently fail.

Vermont's experience across each pillar confirms the framework's structure. The Policy pillar — Acts 167, 51, 68, and the AHEAD agreement — created the mandatory architecture that voluntary reform could not. Without mandatory RBP and mandatory global budgets, Vermont's hospital pricing would continue rising, premiums would continue their decade-long trajectory, and the financial crisis would deepen on the timeline Oliver Wyman projected. Voluntary reform produced OneCare Vermont, which produced modest results among hospitals that chose to participate and no results among those that did not. Mandatory reform changes the calculus for everyone simultaneously.

The Economics pillar confirms that payment architecture shapes behavior more reliably than education, exhortation, or aspiration. When Vermont hospitals operate under global budgets, they will have a direct financial incentive to prevent the hospitalizations that cost money without generating offsetting revenue. When Vermont insurance premiums fall — as RBP and global budget savings flow through to premium rates under Act 68's passthrough requirement — the affordability crisis that has made healthcare the defining economic anxiety of Vermont families will ease. Not immediately. Not completely. But measurably, on a documented timeline, with accountability metrics that allow the legislature to verify progress or demand correction.

The Technology pillar confirms that data infrastructure is not a technical support function — it is the management substrate on which population health management depends. Vermont cannot manage what it cannot measure. VHCURES, for all its limitations, already enables analyses that most states cannot conduct. The CIN analytics platform, the VHCURES-VHIE integration mandate, and the FHIR compliance investments are not investments in technology for its own sake; they are investments in the management capability that global budgets require.

The Clinical pillar confirms what Vermont's own Blueprint data has shown for fifteen years: when primary care is organized as a population health function — team-based, data-driven, SDOH-aware, behaviorally integrated — it produces the utilization reduction and quality improvement that every payment reform is designed to incentivize. The Blueprint's 5.8:1 return on investment is not a projection; it is a documented result from Vermont's own experience. The transformation agenda is, in its clinical dimension, a bet that what works for Blueprint-attributed patients will work for all Vermont patients when primary care investment becomes a system-level commitment rather than a program-level initiative.

The Equity pillar confirms the analytical requirement that transformation cannot be evaluated by averages. Vermont's statewide primary care access rate of 91% — four points above the national benchmark — is a genuine policy achievement that obscures an 11-point gap between white Vermont adults and BIPOC Vermont adults. Vermont's statewide uninsurance rate of 3% obscures an 8% uninsurance rate in Essex County. The transformation that closes the average gap while widening the disparity gap has failed by the standards that Acts 167 and 68 establish. The equity pillar is not a constraint on transformation; it is a definition of what transformation means.

The Operations pillar confirms the gap between strategy and execution that characterizes every healthcare reform effort that has produced documents rather than change. Vermont's transformation will succeed or fail in the operations domain — in whether AHS has the organizational capacity to manage 14 hospital transformation plans simultaneously, in whether the GMCB can design and implement global budget methodology on statutory timelines, in whether the Vermont CIN can build the shared services infrastructure that individual hospitals cannot build alone. The analytical frameworks in this book are necessary but not sufficient. The organizational capacity to execute them is the binding constraint.

"The deepest bet this book makes is about the relationship between analysis and action. Healthcare transformation fails not primarily because of analytical errors — the evidence base for what needs to change is strong and consistent — but because analysis does not automatically produce the institutional capacity, political will, and organizational execution required to implement what the evidence recommends."

— Chapter 3, this volume

Vermont as National Template — The Case for Optimism

The national significance of Vermont's transformation is not self-evident. Vermont is small. Its politics are idiosyncratic. Its hospital system is unusual — 14 non-profit community hospitals, no investor-owned chains, a regulatory body (the GMCB) with authority that most states have never granted any agency. The specific legislative pathway that produced Acts 167 and 68 — the years of community engagement, the Oliver Wyman analysis, the reform cascade structure that built political legitimacy step by step — is not easily replicated in states with more complex political economies, more powerful hospital lobbies, and less institutional trust in state government.

But the structural forces driving Vermont's crisis are universal. The demographic math of an aging population overwhelming a healthcare system built on working-age commercial insurance cross-subsidy applies in Maine and New Hampshire and Vermont and, on a 10-20 year lag, in Ohio and Michigan and Pennsylvania and eventually everywhere. The failure of voluntary payment reform to achieve the tipping point at which fee-for-service becomes the minority mode of payment is a national finding, not a Vermont finding. The administrative complexity that burns out physicians and consumes resources that should go to patient care is a national problem. The equity gaps that persist despite decades of health equity programming are a national pattern.

Vermont's value as a national template is not that other states should copy Vermont's legislation. They should not — the specific provisions of Act 68 reflect Vermont's specific hospital market structure, regulatory history, and political context. The value is that Vermont is generating, in public, with detailed documentation, the evidence base for a set of questions that every state will eventually need to answer: Can mandatory all-payer price regulation reduce hospital prices without hospital closures and access reductions? Can hospital global budgets change hospital behavior toward population health management? Can a state agency restructure itself as a population health system operator? Can a rural state use federal capital investment to build healthcare infrastructure that survives the expiration of the capital investment?

By 2030, Vermont will have answers to these questions. They will not be perfectly clean answers — healthcare systems are too complex for controlled experiments — but they will be the best answers available, grounded in the most comprehensive state-level transformation attempt in American health policy history. Those answers are worth more than any individual policy recommendation, because they are evidence rather than argument, and evidence is what the national conversation desperately needs.

The People This Book Cannot Reach

Every book has an audience it cannot reach, and this one is no different. The people most affected by Vermont's healthcare transformation — the Vermonter in Canaan who cannot afford her insulin, the primary care physician in Newport who is burning out under the weight of documentation, the hospital worker in St. Johnsbury who does not know whether his hospital will still be open in five years, the family in Rutland deciding whether to go to the emergency room or wait until morning because the copay is too high — are not the audience for a six-pillar analytical framework.

This book is written for the people who make the decisions that affect those Vermonters' lives: the legislators who vote on payment reform bills, the regulators who set hospital rates, the hospital executives who decide how to respond to financial pressure, the primary care physicians who choose whether to participate in Blueprint, the AHS staff who design and manage transformation programs. The book's ambition is to give those decision-makers better analytical frameworks, better evidence, and better language for the choices they face.

But the ultimate accountability of healthcare transformation is not to the analytical frameworks that describe it or the advisory practices that support it. It is to the Vermonter in Canaan who should be able to afford insulin, the physician in Newport who should be able to spend her time on patients rather than documentation, the hospital worker in St. Johnsbury who should be able to plan for a career in his community, and the family in Rutland whose healthcare decisions should not be shaped by fear of financial ruin. Those are the people transformation is for. Those are the people who will know whether it worked.

The Work That Remains

Vermont's transformation is incomplete in ways that are analytically important to acknowledge. The AHS restructuring is underway but not finished. The analytics vendor is being procured but has not yet produced the tools that strategic planning requires. The CCBHC network is expanding but has not yet covered all Hospital Service Areas. The GLP-1 BALANCE Model decision has not been made. The Northeast Kingdom equity constraints have been named but not fully addressed in the transformation planning process. The December 2028 Statewide Strategic Plan has not been written.

These are not failures. They are the normal condition of a system in mid-transformation — closer to the destination than to the starting point, but not there yet. The work that remains requires the same combination of analytical rigor, political commitment, and operational discipline that has produced the progress to date. It also requires something that no analytical framework can guarantee: the sustained attention of the people in positions of decision-making authority. Healthcare transformation is slow, and it is easy to let the urgency created by a financial crisis dissipate as the crisis is partially managed and other priorities compete for leadership attention.

Vermont's statutory timeline — the December 2028 Strategic Plan deadline, the mandatory milestones in Act 68, the AHEAD Model's nine-year performance period — is the institutional mechanism for sustaining attention. It converts transformation from a priority that can be deferred to a deadline that cannot. Vermont's legislature, in creating those deadlines, made the most important commitment of the entire reform agenda: the commitment to be held accountable, by specific dates, for specific outcomes. Whether that accountability is actually enforced — whether a Strategic Plan that describes the right questions without committing to specific answers is accepted, or whether the legislature demands the specificity that accountability requires — will determine whether Vermont's transformation produces the system change it promises.

The room in September 2024 was full because people knew something was wrong. This book has tried to give that knowledge a framework, evidence, and language for action. The action belongs to the people in the room.